A/PROF CHAMARA BASNAYAKE

MBBS, PhD, FRACP

Gastroenterologist & Endoscopist

PN 421926PK

FOCUS GASTROENTEROLOGY

20 May 2026

A/Prof Michael Hii
Suite 301, Level 3

Dear A/Prof Hii,

Re: Ms Selma Kahveci
DOB: 10/09/1965 Phone: 0417 569 463
Address: 2 Lovett Drive, AVONDALE HEIGHTS, VIC, 3034
Email: selmakahveci123@gmail.com

Summary

- **Visceral hypersensitivity and central sensitisation following multiple abdominal surgeries.**
- **Commenced low-dose nortriptyline for neuromodulation.**
- **No action required by you.**

I had the pleasure of conducting an in-person consultation with Selma Kahveci with her husband thank you for referring her.

Symptoms and history of presenting complaint

Selma presents with a complex history of chronic abdominal pain. She describes two distinct pain patterns. The first is a constant, low-grade background ache located in the epigastrium and lower abdomen. The second is an acute, sharp exacerbation that is triggered almost immediately (within seconds) by any oral intake, including water. These episodes of worsened pain typically last between 15 minutes and one hour. She notes that the volume or consistency of food does not seem to alter the intensity of the trigger.

Her symptoms have been progressive but worsened significantly following her most recent abdominal surgery approximately one year ago. She has previously trialled sucralfate and esomeprazole without any clinical benefit. She reports her bowel habit is regular and well-formed, with no history of diarrhoea or constipation.

Diet

Selma's diet is relatively limited due to her symptoms. She typically has coffee with dried fruit for breakfast, followed by salads, soup, or fish for lunch, and small portions of meat and vegetables for dinner. She consumes one glass of alcohol a few nights per week.

Past medical history and medications

Selma has an extensive surgical history, beginning with a gastric band 15 years ago. She underwent a gastric bypass three years ago, which has been followed by at least two subsequent operations for internal hernias and an infection. She has also had two ventral hernia repairs, a cholecystectomy (eight years ago), an abdominoplasty, and two emergency Caesarean sections.

Current medications include Lexapro 10mg daily, Mounjaro (commenced four months ago for weight loss), and various vitamin supplements. She has achieved a total weight loss of approximately 100kg over the last

A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

15 years, with her current weight being 64kg.

Social history, family history

Selma is currently not working. There is no family history of inflammatory bowel disease, coeliac disease, or gastrointestinal malignancy.

Examination

On examination, there was evidence of significant abdominal wall hypersensitivity. She demonstrated marked tenderness to even very light palpation (allodynia) across the upper abdomen, which worsened with deeper pressure. I did not identify any palpable masses or evidence of active hernias.

Impression and plan

I agree with you I believe Selma's symptoms are primarily driven by visceral hypersensitivity and a hyper-attuned autonomic nervous system. Given her history of multiple abdominal surgeries, including emergency procedures for internal hernias, it is likely that the neural pathways in her abdomen have become sensitised to any stimulus, such as the stretching of the gut during oral intake or external pressure.

I have explained to Selma that our goal is to "down-regulate" this signalling. I have commenced her on a low dose of nortriptyline (10mg) at night. If she tolerates this without significant side effects (such as morning somnolence), I have advised her to increase the dose to 20mg nocte. I have reassured her that at this very low dose, it will not interfere with her Lexapro, nor is it being used for its antidepressant properties, but rather as a neuromodulator to reset her pain threshold.

We will review her progress in eight weeks. If she reports a 30-50% improvement at that stage, we will consider further titration. I anticipate she may need to remain on this therapy for at least 12 months to achieve a lasting "reset" of the system.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

CC: Dr Ufuk Uluca

All appointments and enquires
Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
Ph: 03 9650 7917
Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065